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A WHITE PAPER

# The Coordination Problem

*Why complex organizations fail in the space no one owns —  
and the architecture that closes it.*

A white paper on institutional blindness and coordination architecture, with complex  
therapeutics as the working example.

*Truth before trajectory.*

LOS ANGELES · AMSTERDAM

A NOTE BEFORE YOU BEGIN

## The things no one owns

Every complex organization contains outcomes that everyone influences and no one truly owns. These outcomes live between functions. Between reporting lines. Between the work people are accountable for and the consequences nobody is.

Most organizations experience these as irritants. This paper argues they are something else entirely.

They are architecture speaking. Specifically, they are where the organization's architecture has stopped matching the complexity of what it is being asked to deliver — and where, consequently, its most consequential failures quietly accumulate, below the reporting, beneath the instrumentation, outside the meetings where performance is discussed.

*Complexity is often only a polite word for unresolved structure.*

That distinction is the subject of this paper. Because if the problem is inherent complexity, an organization accommodates it, plans against it, forecasts around it. If the problem is unresolved structure, the organization can redesign against it — and the performance ceiling the organization has been managing around turns out, on examination, to have been self-imposed.

The paper's working example is complex therapeutics — the categories of medicine, including radioligand therapy, cell therapy, gene therapy, and specialty biologics, in which the delivery of a treatment to a patient requires dozens of coordinated events across multiple institutions, functions, and timelines. The coordination surface in these categories is unusually visible, which makes them useful for showing the pattern clearly.

But the pattern is not confined to complex therapeutics. It exists in any environment where multiple functions pursue a common outcome and the outcome depends on what happens between them. Commercial production. Large-scale manufacturing. Hospital operations. Financial services onboarding. Studio-level media production. Each has its own specific vocabulary and its own specific failure modes. All share the same structural property: the consequential work happens in the spaces between the functions, and those spaces are almost always the least-designed part of the operating model.

This paper names the pattern. It traces why conventional diagnoses miss it, why conventional responses cannot correct it, and what changes when the spaces between are engineered rather than tolerated. It is written as plainly as the subject permits.

It does not contain a playbook. It contains a diagnosis. The playbook, when the diagnosis lands, follows a different conversation.

## I.

### The problem as most organizations see it

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Ask a commercial leader why a complex therapy launch is tracking behind plan and the answers cluster into a familiar shape.

The prescriber base is slower to adopt than modeled. The payer environment is more restrictive than the access team forecast. The site activation timeline slipped because the treatment center's internal readiness was worse than expected. The competitive launch arrived earlier than intelligence suggested. The territory realignment hasn't stabilized. The new medical affairs cadence hasn't produced the expected downstream effect. The team needs more seasoning. The incentive comp may need revisiting.

Each of these answers is defensible. Several of them are often true. All of them together constitute a diagnosis that has never, in the history of complex-therapeutic launches, produced a sustained turnaround when treated as the primary explanation.

Because the diagnosis is downstream.

*Something has gone wrong in the space between the strategy and the patient. The question is what lives in that space, and why no one owns it.*

The organization sets direction. The specialists own their functional lanes — access, medical, clinical education, navigation, supply. The field leaders manage territories. The commercial leadership runs the forecast.

But nobody owns the space between. The patient pathway as a single thing. The referral loop that closes or doesn't. The prior authorization that sits unresolved for seventeen days because two different people each believe the other is working on it. The nuclear pharmacy relationship that is cordial, transactional, and a single personnel change away from collapse. The referring physician who stopped referring four months ago and whose silence has not yet been noticed, because no one's scorecard measures silence.

That space — the in-between, the connective tissue, the places where a complex therapy actually succeeds or fails — is the space where the patient is lost.

It is also the space most pharmaceutical commercial organizations have not yet designed a role to own.

## II.

### The cost of the space between

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The space between the functions is not empty. It is full of cost that no line in any operating plan has been built to track.

The tracking problem is the first cost. A patient who waits three weeks for a prior authorization that should have taken three days is not represented anywhere in the organization's performance reporting as a delayed patient. They are represented as a PA in process. Until, some months later, they are represented as a PA denied, or a patient who never returned, or a referring physician whose referral volume quietly stopped. By the time the cost becomes visible, the patient is gone and the referring physician has already told three of their partners that the pathway is unreliable.

This is what makes coordination failure strategically expensive in a way that conventional commercial scorecards cannot detect. What gets lost is not a prescription but a relationship, and severed relationships take quarters to rebuild — when they can be rebuilt at all.

#### **The compounding**

Coordination failure does not decay linearly. It compounds.

A referring physician who has two patients disappear into an unresponsive pathway does not simply refer two fewer patients next quarter. They refer approximately zero new patients into that pathway, they mention the experience to the colleagues in their practice, and their next referral — if it comes at all — goes to an alternative therapy whose coordination they have learned to trust. The single failed pathway has reset an entire referral community's disposition toward the therapy, not merely one relationship.

A treatment center that has experienced three supply-chain surprises in six months does not simply become more cautious about scheduling. They begin to de-prioritize the therapy in their internal capacity planning. New patient slots go to categories the institution trusts to execute reliably. The therapy is never formally de-listed; it is quietly de-emphasized. The organization discovers this, if it discovers it at all, through a volume decline that appears in the reporting nine to twelve months after the internal shift.

A payer whose denials are not appealed inside the window — because no single person owned the appeal — updates its utilization management model based on observed

behavior. Low-appeal therapies become easier to deny. The PA environment, which the access team was working to improve by policy engagement, worsens through field-level inaction. The policy work continues. The field reality deteriorates underneath it.

The commercial leader reading this paper has probably recognized at least one of these mechanisms already. The referring physician who stopped calling six months ago — you likely know which account that was. The volume dip you explained last quarter as competitive pressure — you likely know which territory that was. The treatment center that used to be your fastest grower and is now quietly plateauing — you likely know which one that is. This paper is not asking you to believe something you have not seen. It is asking you to see the pattern in what you have already been seeing.

Each of these compounding mechanisms operates below the instrumentation. Each produces a result the organization will attribute, in the next quarterly review, to something else — a competitive dynamic, a market shift, a regional anomaly — because the instruments available to leadership cannot distinguish coordination failure from those other causes.

#### **The hidden tax of decision latency**

There is a second cost that travels with the first, and it is less often named. Between the moment structural reality surfaces inside an organization and the moment coordinated action occurs against it, time accumulates. That accumulation is almost never tracked, because no existing instrument measures it directly. But it is, in most complex-therapeutic organizations, the single largest recurring tax on performance.

A prior authorization requirement changes at a specific payer in early March. The field notices, informally, over the following two weeks. The access specialist hears about it from three separate accounts by mid-March. The observation is raised at a regional meeting in early April. It reaches the national access team in late April. A revised appeal template is circulated to the field in mid-May. By the time the organization is executing against the new reality, ten weeks have passed — and during those ten weeks, a specific number of patients did not receive treatment who would otherwise have received it.

That ten-week gap is decision latency, and it is engineered. It is the structural expression of unclear ownership, fragmented signal, and absent interpretive authority. No one in the chain behaved incompetently. Everyone operated within the rhythms of their function. The latency is not what anyone did. It is what no one was designed to do.

Because decision latency rarely appears as a line item, organizations tend to treat it as ambient background — a feature of complexity rather than a product of design. That treatment is the mistake. Latency is diagnosable. It is compressible. And once it is seen as a structural variable rather than an environmental one, it becomes one of the most tractable levers for performance improvement available to a commercial organization.

**The opportunity cost nobody measures**

The most consequential cost is the one that does not appear as a loss at all. It appears as a ceiling.

Every complex therapy has a clinical profile that implies a patient population. The gap between the patient population the therapy could reach and the patient population it actually reaches is the therapy's unrealized opportunity. In categories where the rate-limiting step is coordination rather than science, access, or demand, that gap is often very large — in some categories, the therapy is reaching less than half of the patients its label and payer coverage would support.

An organization that treats coordination as a secondary concern interprets this gap as the natural state of the market. It plans against it. It forecasts around it. It rebuilds its segmentation models to reflect it. It accepts, structurally, the coordination ceiling as though the ceiling were a property of the therapy rather than a property of how the therapy is being delivered.

An organization that treats coordination as the primary engineering problem — that designs the role, the rhythm, and the accountability structure to own the space between — discovers that the ceiling was never the therapy's. It belonged to the coordination architecture. And the ceiling moves when the architecture changes.

*The cost of the space between is not what the organization is losing. It is what the organization has stopped expecting to reach.*

### III.

## The misdiagnosis

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When the pathway breaks, the symptoms appear in the places everyone is watching. Volume softens. Access deteriorates. Engagement slows. Morale dips. The forecast gets rebuilt.

The pattern of organizational response, across many launches and many categories, is consistent enough to be called a reflex. The reflex is to interpret coordination failure as one of three more familiar problems.

The people problem. The team needs upgrading. More talent, more training, more coaching. But replacing people inside an architecture that cannot surface reality or convert it into coordinated action produces the same result — often more slowly, because the new people are still learning it.

The strategy problem. The segmentation was wrong. The targeting missed. The messaging needs a refresh. But a new strategy, layered onto the same coordination architecture that failed to execute the previous one, produces a better-articulated version of the same underperformance.

The resourcing problem. More representatives. More medical liaisons. More specialists. New centralized functions. But adding resources to an architecture that does not yet coordinate its existing resources multiplies friction rather than reducing it — and often introduces additional hand-offs between people who were already not handing off cleanly.

Each of these three explanations is familiar. Each is partially true. Each produces plans, meetings, dashboards, quarterly updates. Each reassures leadership that the problem is being addressed.

None of them addresses the problem.

### **The misread barrier**

There is a related mechanism that explains why the three misdiagnoses above are so durable, even in organizations whose leaders are intelligent, experienced, and actively looking for the real cause of underperformance. The mechanism is the smoke screen — the surface explanation that gets accepted as a driver when the actual driver sits somewhere else.



A physician says the issue is side effects. That may be true. The physician may also be allocating patients into a competing clinical trial and using safety language as a socially acceptable proxy. A treatment center says reimbursement is manageable. That may be true. The same site may also be tolerating margin leakage substantial enough to quietly alter behavior, without having surfaced the economics clearly enough internally to name the leakage as a decision. A stakeholder says they are unconvinced. What deeper examination often reveals is not disbelief but insufficient personal experience, a narrow sample of treated patients, or simple absence of a clinical model anyone has helped them meaningfully consider.

These are not examples of stakeholders being deceptive. They are examples of surface explanations obscuring deeper drivers — and of the commercial organization accepting the surface as the thing to solve.

*A misread barrier produces activity. A correctly understood barrier produces leverage.*

Smoke screens are, by their nature, difficult to see from inside any single function. They reveal themselves only through coordinated perception — across access, medical, field operations, and the treatment-center relationship, held by someone whose job is to see the whole. In most commercial organizations, no such role exists.

Which returns the argument to its starting point. The three misdiagnoses persist because the architecture that would surface their errors has never been built.

*The problem is that no role, no rhythm, and no accountability structure exists for owning the space between the functions. And the space between the functions is where the patient is.*

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## IV.

### The unit of analysis

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Every commercial organization chooses, explicitly or implicitly, a unit of analysis. The thing it measures. The thing its scorecards score. The thing around which its meetings are built. The thing it optimizes.

The traditional pharmaceutical unit of analysis is the prescriber. This made sense when the therapies were simpler, the patient journey was shorter, and the prescribing decision was the rate-limiting step. The model — identify high-prescribing physicians, detail them, track their prescribing, compensate accordingly — was internally coherent and demonstrably effective for the category of problem it was designed against.

Complex therapeutics are a different category of problem.

In a radioligand therapy program, the prescribing decision is one event in a sequence of forty. The referring oncologist identifies the patient. The patient has to be clinically characterized, often with a specialized imaging modality that requires its own referral and its own scheduling. The treatment center has to confirm eligibility, coordinate with the nuclear pharmacy, secure the therapeutic dose on a supply chain that operates on a half-life measured in hours, obtain prior authorization from a payer whose requirements may have changed since the last patient, schedule the patient across a multi-cycle regimen, deliver the therapy inside a radiation-safety protocol, manage the post-treatment monitoring, and communicate outcomes back to the referring oncologist — who will, or will not, continue referring based on whether that loop closed.

The prescribing decision sits somewhere in the middle of that chain, and it is not the rate-limiting step. The rate-limiting step is whichever of the forty events is currently breaking.

Radioligand therapy is this paper's working example because the coordination surface is unusually visible — the half-life clock, the nuclear pharmacy, the radiation-safety protocol — but the pattern extends well beyond it. In autologous cell therapy, the pathway includes apheresis scheduling, manufacturing slot allocation, lymphodepletion timing, bridging therapy, cytokine release monitoring, and a reimbursement architecture the treatment center is rarely prepared for. In gene therapy, it includes germline counseling, long-term follow-up commitments, and a payer dialogue that can extend across years. In specialty surgical implants, it includes device availability, OR coordination, rehabilitation protocols, and outcomes reporting. Each of these categories has its own specific chain. All of them

share the same structural property: the prescribing decision is one node among many, and the decisive constraint is operational.

### The pathway, not the account

This is why a commercial model built around the prescriber produces results that do not match the opportunity the therapy’s clinical profile would predict. The model is measuring the wrong thing. It is counting calls on physicians when the constraint is sitting in a scheduling coordinator’s inbox.

The unit of analysis, for any complex therapy, has to be the patient pathway. From identification to completed treatment cycle. As a single object. With a single owner.

The pathway is what matters, not the account. An account can be thriving on every conventional metric — access approved, physicians engaged, protocols in place — while patients are still being lost to friction that doesn’t appear on any account-level scorecard. Patients wait. Referrals stall. Specialists operate in isolation. And the organization mistakes a coordination failure for a market access problem.

FIGURE 1  
The patient pathway, as the unit of analysis  
WHAT THE CONVENTIONAL MODEL OPTIMIZES

*identification · referral · imaging · characterization · eligibility · [ PRESCRIBING DECISION ] · prior auth · supply chain · scheduling · site readiness · infusion · loop closed*

PATIENT IDENTIFIED → PATIENT TREATED

#### WHERE PATIENTS ARE ACTUALLY LOST

*The prescribing decision is one event in a sequence of many. The rate-limiting step is whichever one is currently breaking.*

When the pathway is the unit of analysis, the measurement changes, the decision rights change, and — most consequentially — the role of the field person changes. They are no longer running a territory. They are running a pathway, across multiple accounts, with responsibility for what moves through it.

This is an architectural shift rather than a semantic one. It reorganizes what the field person is responsible for, what they are measured on, what authority they carry, and how the specialist resources around them are deployed. It has downstream implications for org design, compensation, performance management, and leadership reporting — implications this paper will trace but not fully publish.

## V.

### A fourth category of commercial work

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Commercial organizations in complex-therapeutic categories have, historically, organized their work into three legs. There is access work — payer policy, coverage, coding, reimbursement. There is medical work — scientific exchange, clinical education, evidence generation. There is commercial work — identification, targeting, messaging, the promotional interaction and its downstream measurement. These three legs are well-understood, well-instrumented, and correctly resourced. The roles that execute them have titles, scorecards, career tracks, and compliance frameworks that have been refined over decades.

There is a fourth leg. It has not yet been named.

It is the work of owning the pathway a patient moves through from identification to completed treatment. It is coordination work — not in the clerical sense of tracking, but in the operational sense of being the person responsible for whether the pathway actually runs. It is relational work — the patient-weighted trust that a treatment center, a referring community, an advocacy ecosystem, and a specialist team all place in a single operator who they know will answer when the pathway breaks. It is diagnostic work — the field-based sensing of where in the pathway friction is accumulating and what specialist capacity needs to be deployed against it.

This fourth leg is a distinct category of work, with its own logic, its own measurement frame, and its own compliance profile. An organization that treats it as an extension of the commercial leg will find that the compliance boundary forbids most of what the work requires. An organization that treats it as an extension of the medical leg will find that the scientific exchange framework does not support the operational authority the work demands. An organization that treats it as an extension of the access leg will find that payer navigation, while necessary, is only one of forty things the work has to coordinate.

The category needs a name and a role to execute it. This work is ecosystem coordination, and the role that owns it is the Ecosystem Strategy Lead. Other organizations may arrive at other names. The naming matters less than the recognition that the work is a fourth category, equal in weight to the existing three, and that it is the category whose absence explains most of what the other three have been asked — and have been unable — to fix.

## VI.

### The operator the category requires

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Most complex-therapeutic commercial organizations have a role that approximately resembles the one this category requires. It is usually called a key account manager, a regional strategy lead, an ecosystem director, or some hybrid with a similarly ambitious title.

The title is usually accurate to the intent. The role, in practice, almost never is. Because the role is typically sized, structured, and compensated in a way that requires the person holding it to behave, most of the time, like a territory manager with broader geography. Volume targets remain. Quarterly forecasts remain. The reporting cadence runs on weekly call plans and account plans, not on pathway friction and constraint resolution. The person in the role cannot be what the org chart says they are, because nothing around them has been reorganized to let them.

The role the pathway requires is defined not by what it reports to or what it promotes, but by what it is the first call for.

#### **The standard, not the title**

The Ecosystem Strategy Lead is where ambiguity ends.

They are field-based. Not as a commitment to presence theater, but because the signal that determines whether a pathway is healthy is not available anywhere else. The navigator who looks tired. The referral coordinator who hesitates before answering. The treatment center director who mentions offhandedly that another therapy is gaining traction with a physician who was supposed to be locked. None of this appears on a dashboard. All of it matters. The ESL collects it by being physically present, twice a week minimum, in the places where the pathway either succeeds or fails.

They are relationship-sovereign. The treatment center does not call the home office when something breaks. They call the ESL — because the ESL has made themselves the first call by being the person who answers, who acts, and who follows through every single time. That reputation is built visit by visit, problem by problem, Thursday by Thursday. It is the ESL's primary asset. It is built in the field. It cannot be built on a Zoom call, maintained by email, delegated to a specialist, or generated by a campaign.

They are strategically indispensable. When they leave the room, the coordination does not continue to run. The account that tries to move without them discovers quickly that things move slower, break more often, and cost more to fix. The ESL is indispensable not because they are talented, although they are, but because the architecture of how coordination happens around them routes through them on purpose.

#### **What the ESL is not**

It is easier to describe what the role is by describing what it is not.

- **Not a coordinator with a better title.** A coordinator tracks. The ESL owns the outcome of what is being tracked, and the tracking is an artifact, not the work.
- **Not the motivator.** Performance in complex-therapeutic coordination is not motivation-limited. It is architecture-limited. An ESL who substitutes motivational effort for structural clarity is doing the wrong job well.
- **Not the meeting chair.** The meeting is an instrument, not an identity. A role whose primary function is to run meetings has been mis-scoped.
- **Not the smartest person in the room.** They are, more precisely, the person who is most clearly in service of the pathway — a different quality, and often more useful than intelligence.
- **Not promotional.** The ESL does not generate demand. They generate proximity. Between the patient and the pathway. Between the strategy and the execution. Between the specialist's capability and the account's specific need at this specific moment.

This last distinction matters more than the others, because it determines what is possible inside the compliance boundary that governs every interaction a pharmaceutical field person has with a customer. The ESL can coordinate, support, escalate, and close loops because none of those activities are commercial asks. They are operational support. A pathway that is coordinated, supported, escalated, and closed produces referring physicians who continue referring — not because they were asked to, but because the experience of having referred was good enough that the next referral is the natural clinical decision.

A role designed around the commercial ask cannot do this work. A role designed around the pathway can.

## VII.

### The architecture that surrounds the role

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The ESL does not operate in a vacuum. A person with the right standard, placed in an organization without the right architecture, becomes exhausting to employ and impossible to retain. They succeed for a while on individual capability, then plateau, then burn out, then leave — and the organization concludes that the role was never viable, when in fact the role was never supported.

The architecture that makes the role viable has six layers. This paper will describe the purpose of each. It will not publish the operating mechanism.

#### **Layer one — Identity**

How the role shows up. The standard of daily behavior that separates an ESL from a senior coordinator is not a set of competencies or a list of values. It is a small number of non-negotiable daily postures — humility, hunger, hard work, honesty, and a radical ownership of outcomes — rendered into behaviors that are observable, correctable, and replicable. The identity layer is where the role's credibility is built, day by day, in front of the people whose trust the role depends on.

#### **Layer two — Signal**

How truth surfaces. Every complex organization has a quiet version of the same failure: the truth about what is actually happening does not reach the person who needs it in time to act on it. Either because no structural moment exists for it to be told, or because the structural moments that exist have been captured by the instinct to solve or defend or reassure. The signal layer is a purpose-built structural moment in which the only activity permitted is observation. What changed this week. Where patients are stuck. Who is engaging differently. What feels harder than it should. Captured without solution, without defense, without hierarchy. The rules of this moment are specific and non-negotiable, and they separate an organization that can see reality from one that cannot.

#### **Layer three — Synthesis**

How decisions get made from signal. Signal alone is not a decision. Signal gathered in the presence of a group almost never becomes a decision, because groups decide by negotiation and synthesis is not a negotiation. Synthesis is the act of one operator — the ESL — sitting alone with the week's signal and producing, from it, a small number of things. What is actually happening. What the constraints are. Which specialist deployment the

constraints require. Which account tier moved and why. The synthesis is the most intellectually demanding work the ESL does. It is also the most private. It is not a meeting.

#### **Layer four — Rhythm**

When things happen. Predictability is the infrastructure of trust. A team that does not know when signal will be surfaced, when synthesis will be delivered, when deployment will be briefed, and when the scorecard will be read cannot make plans around the work. Rhythm is a structural device rather than a calendar convenience. It lets the field team, the specialist team, and leadership all act from the same assumptions about when decisions happen. When the rhythm breaks, people fill the gap with their own assumptions, and coordination collapses into the individual effort it was built to replace.

#### **Layer five — Accountability**

Why people care. Visibility — honest, unsoftened visibility — is the mechanism. When the scorecard reflects reality without flattering it, accountability is automatic. When the scorecard is edited to protect the organization's comfort, accountability becomes a political activity. The question of whether an organization can tolerate the first kind of scorecard is the single most predictive question about whether this architecture will produce what it is capable of producing. It is also a question most organizations answer in the abstract far more confidently than they answer in practice.

#### **Layer six — Safety**

Why people stay. A high-demand system without psychological safety produces attrition. The safety layer is what makes the demand sustainable. It does this through specific mechanisms — normalized early flagging, modeled vulnerability from the ESL, negotiated ownership rather than assigned blame, and periodic formal review points at which the relationship between the individual and the system is re-contracted rather than left ambient. The safety layer is what distinguishes an operating system that demands high performance from one that merely punishes its absence.

*Identity, signal, synthesis, rhythm, accountability, safety. Six layers, in sequence — because each depends on the one before it.*

Each of these layers is necessary. Skipping any one of them does not weaken the architecture proportionally. It collapses the architecture entirely, because the layers are interlocking, not additive.



The mechanisms inside each layer — the rules of the signal moment, the structure of the synthesis, the operating rhythm, the design of the scorecard, the specific safety mechanisms — are not the subject of this paper. They are the subject of the engagement that follows.

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## VIII.

### The counterpart inside the institution

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There is a limit to what any external role can accomplish.

The ESL owns the pathway from outside the treatment center. They know every person in the institution who touches it — the director, the coordinator, the pharmacist, the scheduler, the navigator — by name, by nature, and by what they need before they ask. They are present. They answer when it breaks. They follow through.

But the treatment center has an inside. And inside the treatment center, every day, things happen that no external role can see in time to act on. A scheduling conflict that takes two weeks to resolve because the people who need to coordinate sit in different reporting lines. A supply chain anxiety that the pharmacy director is carrying alone because the internal structure has no forum for surfacing it. A miscommunication between clinical staff that collapses a pathway the ESL has spent months building from outside.

The external role, however well-executed, cannot solve for the inside. The moment the ESL tries to own both sides of the institutional door, their field presence across multiple accounts degrades, and the pathway loses its most consequential external monitor at exactly the point it needs them most.

#### **The Internal Pathway Coordinator**

The solution is to give the treatment center the structural equivalent of an ESL inside their own walls.

In most treatment centers, this person already exists in a related capacity — a senior treatment coordinator, a program navigator, an oncology operations lead. The role is formalized by the hospital, designated by the hospital, and paired with the ESL as a peer operating partnership. The ESL watches the external pathway and deploys resources against external friction. The IPC watches the internal pathway and surfaces internal friction before it reaches the patient.

Together, they close the entire pathway — as a peer operating partnership where each person owns their side of the same outcome, not as a hierarchy.

The IPC is an institutional role, not a pharmaceutical one. The hospital designates the person. The hospital defines the role's reporting line. The hospital retains full ownership. What the firm contributes is the consultative work of framing the value proposition for the

hospital, defining the operating relationship with the ESL, and integrating the IPC into the operating rhythm of the pathway support team.

*The hospital cannot run the internal pathway without the IPC. The IPC cannot access specialist resources without the ESL. The ESL cannot maintain real-time internal intelligence without the IPC. The system has made three parties mutually dependent in service of one outcome.*

This is the most underdeveloped structural relationship in contemporary complex-therapeutic commercialization. It is also the one with the most leverage. A pharmaceutical field model that has built the IPC partnership at every account in its geography is building infrastructure, not relationships. And infrastructure, once built, is the most durable form of indispensability available to a field organization that is serious about what it is trying to produce.

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## IX.

### When this architecture does not work

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A paper that is honest about what a method can produce must be equally honest about the conditions under which the method produces nothing.

This architecture does not work when leadership prioritizes optics over truth. The scorecard's only value is that it reflects reality. An organization that requires the scorecard to flatter — for quarterly review reasons, for cross-functional peace reasons, for career management reasons — has decided it does not actually want the instrument. The architecture can be installed in such an organization, but it will be gradually reshaped into a version of itself that no longer produces its intended effect. This is a decision about what the organization wants, not a failure of the architecture.

It does not work when field presence cannot be structurally protected. The ESL who is in the field two days a week is indispensable. The ESL who has been pulled into Zoom calls, internal meetings, and administrative rhythms to the point that their field presence has compressed to a half-day is a coordinator with an aspirational title. The organization that cannot protect the field days — not in principle, but in specific structural ways that withstand the weekly pressure to reallocate time — has not yet committed to the premise that the signal and the relationships on which the role depends live where the patient is.

It does not work when cross-functional participation is performative. The signal moment works only if the access specialist, the medical liaison, the pharmacy liaison, the navigator, and the operational lead are contributing observations from their functional vantage point without filtering them through what solution it might create. An organization in which the specialists attend the signal moment but do not participate in the signal — because their reporting lines penalize them for surfacing what they actually see — has installed the ritual without building the architecture.

It does not work when transparency is perceived as threat. A system that surfaces reality inevitably surfaces things that are uncomfortable for specific people in specific roles. If the organization's response to discomfort is to suppress the instrument that produced it, the instrument will be suppressed. Whether an organization can tolerate the discomfort of its own truth is an operational question, not a philosophical one, and it determines outcomes.

It does not work when the organization is unwilling to tolerate short-term discomfort in exchange for durable performance. This architecture, installed correctly, produces a

period of visible friction before it produces visible improvement. Problems that were previously invisible become visible. Individuals who were previously protected by the opacity of the old system become exposed by the clarity of the new one. Leadership that cannot absorb that period — that intervenes to soften the instrument before it has produced its first cycle of corrective effect — gets the discomfort without the benefit.

*These conditions are not managed around. They are named. Directly.  
Specifically. Without apology.*

This is a consequential decision about how the practice operates, and it is worth making explicit. A consulting engagement that proceeds into an environment that cannot support the work it is delivering produces a predictable sequence: the work is installed; the organization's existing reflexes slowly reshape it; the installed version of the work stops producing its intended effect; the consulting firm either pretends not to notice, or notices and doesn't say anything, or raises the concern and is quietly distanced from the account; the work is eventually disassembled and replaced by the next initiative; and everyone involved tells themselves that the model didn't quite fit the organization.

Declining the engagement is preferable to participating in that sequence. The pre-engagement conversation includes a small number of direct questions about the conditions above, and the quality of the answers to those questions — more than any other input — determines whether the engagement proceeds.

An organization that cannot answer them cleanly has not been lost. It has been correctly declined.

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## X.

### What the world looks like when the pathway is owned

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It is one thing to describe what is missing. It is a harder and more consequential thing to describe what is present when the missing thing is installed.

An organization that has built the architecture this paper has described does not look, from the outside, like an organization that has installed something dramatic. The dramatic signals of conventional commercial intervention — the campaign relaunches, the realignments, the offsites, the new incentive plans — are largely absent. What is present instead is a quieter set of operational facts that compound, week by week, into a materially different outcome.

#### **Monday morning**

The operator who owns the pathway has already spent most of the prior week physically present at the places where the pathway succeeds or fails. They walk into Monday's signal session carrying observations that no report could have produced: a navigator who mentioned, offhand, that a specific payer's prior authorization checklist has changed; a pharmacy director who is quietly anxious about a supply window in two weeks; a referring oncologist whose nurse said something in passing about another therapy's outreach that did not happen a month ago. The access specialist, the medical liaison, the pharmacy liaison, and the navigator all arrive carrying their own week's observations from their own vantage points. The session captures all of it — messily, without solving, without defending — in under an hour.

By Monday afternoon the organization has a fuller picture of what is actually happening in the field than the combined reports of the previous quarter produced.

#### **Tuesday**

The synthesis happens quietly. One operator, working alone with the week's signal, identifies the two or three constraints that actually matter and decides which specialist deployments the constraints require. This work is slow, private, and inexpensive to the rest of the organization. It produces a small number of decisions that are consequential and a large number of observations that are filed for future reference. The decisions do not require leadership approval to execute. They require only that the specialists be available for deployment on the right day to the right account, which is what the rhythm exists to guarantee.

**Wednesday and Thursday**

Specialists deploy with full context. A Reimbursement and Access Specialist does not arrive at a treatment center to introduce themselves and ask what is needed. They arrive knowing that a specific payer has changed a specific requirement, that three specific patients are caught in the resulting backlog, and that the revenue cycle manager they are meeting has been waiting, since Monday, for someone to help them clear the backlog. The interaction takes forty minutes and produces a resolution. In a conventional model, it would have taken three weeks and four rounds of email, if it had happened at all.

The operator is in the field. They are at the treatment center that flagged a concern on Monday, at the referring practice whose silence had been noticed, at the advocacy group whose relationship has been deepening quietly over months. The pathway is being monitored by the person for whom monitoring it is the job, not an activity added onto other jobs.

**The downstream effect**

A month of this rhythm produces effects that are difficult to attribute to any single intervention but are visible across the pathway. Prior authorization cycle times decline because appeals are being initiated inside the window and by people who have been briefed. Referring physicians who had quietly stopped referring are, in some cases, referring again — not because they were asked to, but because a loop that had been left open was closed. Treatment centers that had been treating the therapy as a category to de-prioritize begin to treat it as a category they can rely on, because the pattern of their own experience has shifted. The coordination ceiling that the organization had previously planned against begins to move, by small amounts, in the right direction.

A quarter of this rhythm produces effects that are visible in the conventional reporting. Volume improves. Dropout declines. Access metrics stabilize. These improvements appear, to leadership, as though the commercial trajectory has finally turned — which it has, although not because of any of the activities that conventional commercial reporting would identify as the cause.

The cause is the coordination architecture. The architecture makes the pathway visible to the people who can move it. The visibility makes the friction surface early enough to act on. The action, routed through a single accountable operator with a deployed specialist team, produces resolution at the velocity the patient's clinical situation requires. The patient gets treated. The referring physician continues referring. The treatment center continues

scheduling. The advocacy group continues trusting. And the therapy reaches a population that, under the previous coordination architecture, it was never quite reaching.

*None of this is dramatic. All of it compounds. And compounding, over the quarters that matter, is what distinguishes a therapy that achieves its clinical potential from one that does not.*

This is the operating reality the architecture produces. Not a better version of the current reality but a different one — visible to leadership through the same reporting systems, yet built on an entirely different set of underlying mechanics.

Organizations that have not seen this operating reality tend to assume it is aspirational. Organizations that have seen it describe it, consistently, as the first time in their complex-therapeutic career that they felt the commercial system was operating on purpose rather than on effort.

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## XI.

### What this paper has been about

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This paper has described a coordination architecture. It has used pharmaceutical complex therapeutics as its working example. The architecture is real, specific, and proprietary in its operating details, and complex therapeutics is the category in which this practice is most sharply positioned to help.

But the thing the paper is actually about is older and larger than any single industry.

It is about what happens inside complex organizations when the work has outgrown the architecture that once contained it. It is about outcomes that everyone influences and no one owns. It is about the ceilings that look like market reality but are, on examination, structural. It is about the smoke screens that produce activity instead of leverage, the decision latency that taxes performance below the instrumentation, the misdiagnoses that sophisticated organizations cycle through while the underlying gap remains unnamed.

It is, in short, about the places where an organization's architecture has stopped matching the complexity of what it is being asked to deliver.

#### **Four questions**

Most of what this paper has argued can be reduced to four questions. They are not rhetorical. They are diagnostic. A leader reading this paper can use them — now, or later, or in the next quarterly review — to surface whether the patterns described in these pages are present in their own organization.

The first: are we managing around ceilings we have never independently verified are real?

The second: where, specifically, are the outcomes that everyone influences and no one owns — and what is that unowned-ness costing us?

The third: are we accepting stated explanations as causes, or are we surfacing the drivers underneath? When a physician says the issue is side effects, when a site says reimbursement is manageable, when a stakeholder says they are unconvinced — what is actually happening?

The fourth: what is the time between when reality surfaces somewhere in our system and when we act on it — and what is that latency costing us, in quarters we cannot get back?

These four questions will not produce a strategy. They will produce a first honest look at where the organization's interpretation of itself may have drifted from its actual operating reality. That first look is where the real work begins.

**The patient at the end**

The paper's argument has been architectural, systemic, and — at times — abstract. The thing that makes any of it worth doing is neither.

Behind every system that runs. Behind every specialist who deploys with precision. Behind every referring physician who keeps referring because the loop keeps closing. Behind every treatment center that calls someone first because they know that someone will answer. Behind every advocacy group that trusts the pathway because someone showed up when it mattered.

There is a patient who got treated.

That is the only scoreboard that matters. The architecture exists to make that scoreboard move. In other industries the scoreboard will look different — a product that shipped on time, a patient who recovered, a transaction that closed, a project that delivered to its brief — but the principle holds. The work of the practice is not to be admired. It is to produce an outcome that a real person, somewhere at the end of the chain, is waiting for.

The organizations that build the architecture to produce those outcomes reliably are not more heroic than the ones that do not. They are simply more structurally honest about what the work requires. That honesty is what this paper has been, at its foundation, asking for.

**If the diagnosis has landed**

If you have read to this point, you have probably recognized part of your own organization in the argument. That recognition is not a conclusion. It is a starting point.

Leaders who have engaged this practice typically describe the experience of reading something like this paper as the moment they were finally given language for what they had been carrying for some time — a sense that the structure of their work had stopped matching the complexity of what their work was being asked to deliver, without a clear way to name it or to act on it.

If that is a familiar feeling, the next step is a single conversation. Not a proposal. Not a pitch. A ninety-minute call spent listening carefully to the specific situation the organization is in,

asking a small number of direct questions about the conditions around it, and saying — honestly — whether an engagement would be appropriate, whether the conditions would need to change before it could be, or whether another firm would be a better fit for the situation.

This practice engages three to five clients per year. This is not a marketing claim. It is the operating condition that makes the work possible. Decisions about a given year's engagements are typically made in the first quarter.

If the diagnosis in this paper has landed — if the four questions above produced a moment of recognition rather than deflection — the conversation begins with one email to the address on the final page.

**Truth before trajectory**

The trajectory of an organization is built, in the end, on what the organization has been willing to see. This paper has been about what most organizations, most of the time, have not yet been given the language or the architecture to see.

The rest is a conversation.

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### *A note on this paper*

This is a diagnostic paper. It describes the shape of a method, not the mechanism of one. The operating architecture — the rules of the signal moment, the structure of the synthesis, the design of the scorecard, the specific safety mechanisms, the system integrity function — is the work this practice is engaged to install, and is developed inside each engagement against the specific conditions of the organization. It is not published.

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